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EQUALITY, EQUITY AND HEALTH POLICY | REVIEW ARTICLE

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More than an ally: becoming an accomplice to cultural safety for Australian First Nations people and Indigenous knowledge

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ABSTRACT

Introduction: Culturally safe practice is a foundational requirement of all Australian health professionals and is now mandated within their codes of practice. Being a culturally safe practitioner requires understanding of the concepts of social justice, equity, and the principles of respectful engagement with diverse groups of people.

Theory: There are fundamental similarities applying to equity for all people, and specific requirements for diverse groups. Here we focus on respectful and effective engagement with Australian First Nations people, who have the most concerning outcomes in health. Many health practitioners are open to becoming accomplices to First Nations peoples' struggle for equity, self-determination and respect for Indigenous knowledges, but are unsure how to begin.

Methods/Results: We invite all to begin their journey from allyship towards accompliceship and culturally safer practice. To do this we offer pragmatic guidance and examples. Our four guiding principles contain practical skills to inspire others to begin and continue their own journey towards accompliceship.

Discussion: We offer a grounded approach, to expand understanding of how discrimination operates, and how practical equity for First Nations people can be enacted every day. In a world where discrimination of difference has been normalised, becoming an accomplice is a contribution to a better world for all.

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Accomplice; accompliceship; First Nations people; cultural safety; equity health and fitness; whiteness

Introduction

Practicing cultural safety as health professionals requires an understanding of the social, cultural, historical and systemic influences upon lived experience and outcomes for First Nations peoples. These influences include cultural protocols, support structures, and multiple systemic barriers of disadvantage and discrimination, both obvious and subtle. Accompliceship emerges through incorporating this understanding into equitable, meaningful actions within everyday practice. Our focus is Australian First Nations peoples, and we are aware there are some similarities for First Nations peoples in other parts of the world. The term accomplice is used to denote intentional affirmative action (Bruno, 2023; Parter et al., 2023). We use the term accompliceship instead of ally, as the latter can be claimed without practical action being taken or change occurring (Finlay, 2020; Parter et al., 2023). Accomplices stand alongside First Nations peoples in their struggle for social justice and a voice in policies that directly impact their people (Finlay, 2020; Southam, 2021).

The term equity needs qualifying here, as it can be used in a way that obscures lack of cultural safety or indeed equitable outcomes. Given the dominance of western knowledge and cultures in healthcare, have you ever asked yourself 'equity from whose perspective'? Equity cannot exist without acknowledging and addressing power imbalances and discriminatory practices within health institutions and social and cultural structures (Curtis et al., 2019). The definition of equity applied here includes the fact that conditions of privilege or discrimination are not necessarily predictive or causative of specific health

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outcomes but correlated through ongoing inequity within institutional structures and professional practice. Authentic equity includes voice, access, participation, and rights (Burton et al., 2024; United Nations, 2008). Accompliceship to First Nations people enacts authentic *practical equity* through everyday actions and relationships (Southam, 2021).

Processes and practices that enable racist and discriminatory actions remain embedded within healthcare, despite being unethical and outdated (Curtis et al., 2019). However, many institutions have policy requirements and guidelines for respectful professional practice when working with vulnerable minority groups. Cultural safety is a professional and legal requirement in all healthcare professions (Australian Health Practitioner Regulation Agency & National Boards, 2020). Not a mere box ticking exercise, but as enacted reality. Cultural safety is defined in Australia as 'the ongoing critical reflection of health practitioner knowledge, skills, attitudes, practising behaviours and power differentials in delivering safe, accessible and responsive healthcare free of racism' (Australian Health Practitioner Regulation Agency & National Boards, 2020). Each of us working in healthcare bears responsibility to integrate equity and fairness into our practice. Some take up this challenge through daily actions of respectful care and consideration. Others may contribute to institutional policy, calling out discriminatory protocols and practices.

We all make a difference. Accessing healthcare forces First Nations people to interact with western institutions and everything we do has positive or negative effects. Health practitioners in every role are either contributing to equity in service provision or are complicit in inequity and discrimination, via their silence, inaction, or worse (Schmidt, 2019). We cannot sit on the fence imagining we are neutral and somehow outside of the reality of treatment and outcomes for First Nations people.

Social and practical justice are manifestations of fundamental human rights (United Nations, 2008). Racism and discrimination towards *any* minority have no place in healthcare. Instead, ethics of dignity, respect and fair treatment create conditions for improving health outcomes for those with the worst statistics and outcomes in Australia. As health settings become increasingly diverse, culturally safe professional skills are crucial. Not just in relationship with First Nations peoples, for all minorities. This includes wellbeing in the workplace for both recipients of care *and* practitioners. Although practitioner wellbeing is not the focus of this article, workplace discrimination in the form of undermining or bullying through sexism, homophobia and racism are also manifestations of wider prejudice and cannot be ignored. Discrimination experienced by staff who are not members of the dominant cultural group, as well as patients, are common everyday experiences in Australia (Bargaille, 2020). This has increased from 50% to 59% since the 'NO' campaign's misinformation and social media blitz contributed to Australians' rejecting recognition of First Nations peoples in Australia's constitution in 2023 (Diversity Council of Australia, 2023). Being fully informed on rights to respect at work and communicating respectfully within institutional cultures are crucial to the skillset required of an accomplice to First Nations people (Parter et al., 2023).

When we stand up as accomplices to equity, there can be pushback from people who are benefiting from the whiteness myth of superiority. Excluding, supressing and discriminating against some people and acting in a destructive way towards alternative worldviews and lifestyles is a recipe for social disaster, as many of the challenges of the 21st century attest. Enabling all people to participate fully in their communities is a necessary condition for societal health and coherence, aligning with the United Nations Declaration on the Rights of Indigenous Peoples (United Nations, 2008). We note that at this time in history, there is a groundswell of conservativism seeking to maintain/re-enshrine discriminatory policy and practices. Now more than ever, cultural safety for every person we encounter as a health professional is an act of humanitarian resolve; an act of deciding to hold respect and care. In these times we all need to hold a space of unbiased professional practice.

We offer four guiding principles that open a pathway to becoming an accomplice. These principles suggest points for exploration, practical steps and crucially, self-reflection. We offer anecdotes and stories from our own professional experiences. We provide a foundation for building practitioner confidence to begin their journey towards accompliceship. Towards authentic care and consideration of the needs of First Nations peoples and other minority groups. These principles are transferable to all people or groups we engage (Came & Griffith, 2017).

Being an effective accomplice is a lifetime journey of unlearning for white Australians (Rix et al., 2024), as we challenge views that have been taken for granted for centuries. For example, the default position

within healthcare, that western and biomedical knowledges are the 'gold standard,' relegates non-western cultures and knowledges to the status of lesser, irrelevant, or simply the 'other.' Providing 'equity' in healthcare requires respectful inclusion of diverse ways of being and knowing. Not to replace western knowledges and practices, rather to enhance and increase effective care. We are not claiming absolute or fixed solutions. Rather, letting go of thinking *any group* has *all* the answers, opening ourselves to lifelong learning, can contribute to improvements (Bruno, 2023). Our four principles are: (1) Reality is complex, (2) Ready to learn, (3) Action and being brave, and (4) Reflection.

Firstly, the foundations of whiteness and intersectionality, and 'nothing about us without us,' are laid.

Whiteness and intersectionality

The terms white and whiteness are not used to directly denote skin tone, but as a signifier for structures of power and institutional systems of discrimination where they are enacted by individuals and groups holding power. The terms came about because those who enact or benefit from whiteness have historically been of European/Caucasian descent. This situation is an ongoing structure of discrimination according to Mackay et al.: 'Whiteness and institutionalised racism began in Australia in 1788 and continues to this day in many aspects of our culture and society' (Mackay et al., 2023, p. 6), operating within a hierarchy of automatically assumed superiority, based on appearance and assumptions. People of many cultural backgrounds may be living/working within, perpetuating, and benefiting from western/white systems and institutions. The way we use the term whiteness draws on extensive research and literature into the enormity and pervasiveness of this structural and cultural system of colonialism, dominance and discrimination (Bonds & Inwood, 2016; Mackay et al., 2023; McIntosh, 1990; Mitchell et al., 2022). It also relates to the notion of white fragility, that is the often-emotional, victim-oriented reactions of those facing challenges to their participation in and benefiting from these structures of power and discrimination (Diangelo, 2018).

Nothing about us without us

The voices of First Nations people must be sought when policies and practice affect their lives. To not do so is patronising and insulting. Healthcare institutions have not traditionally sought the opinions of those accessing services, yet the time taken to engage in authentic communication/consultation have positive effects on health outcomes. The phrase 'Nothing about us without us' originated from Charlton's (1998) book of the same name. While Charlton's work relates to disability rights, the phrase's meaning is transferrable to all minority groups experiencing discrimination. Protocols can be created to streamline a process of effective, authentic and respectful consultation. Like many aspects of accompliceship, this is not about getting it perfect, feedback and adjustment will support refinement. Not doing anything is the only way to get it wrong.

The central importance of this ethical and respectful perspective is demonstrated by the development of this article. Authors Dr Elizabeth Rix and Dr Shae L Brown developed the draft principles, with consultation with First Nations scholar, health professional and co-author Dr Darlene Rotumah refining and validating the authenticity of our principles, and the veracity of our approach to accompliceship. This collaboration demonstrates the necessity of stepping up as non-Indigenous professionals, with openness, willingness and critical reflexivity, to listen deeply, and learn from First Nations experts.

Guiding principles of First Nations accompliceship

Our four guiding principles offer pragmatic strategies for a lifelong journey of accompliceship to practical equity. These principles demonstrate relationship, respect, responsibility and reciprocity (Tsosie et al., 2022). These four 'Rs' are the fundamental attributes of accompliceship with First Nations people, with relevance to any minority group experiencing discrimination (National Health & Medical Research Council, 2018). The principles are interconnected and overlapping; however, it is useful to have guidance in the form of specifically focused points. They form a lens, enabling us to bring the complexities of human life into focus, so we act with broader vision (Brown, 2021). Thinking with a complexity perspective helps us to see wider

ranging factors that influence any situation. These can be factors that privilege or discriminate (Brown, 2021). Thinking this way assists us to connect with the whole person placed within our care (Gatwiri et al., 2021).

> Reflecting on my role as an Aboriginal health professional and the experiences I've had over the years, I'm reminded that our people depend heavily on spaces that feel safe, welcoming, and culturally grounded. I've always felt the weight of that, knowing that Aboriginal health workers are often the ones breaking down barriers and creating that sense of safety, even though our workforce is small and stretched thin (Rotumah, 2023).
>
> I've seen moments where care is respectful and centred on relationships, and those moments remind me why this work matters. But I've also seen the opposite, times when the interaction isn't safe, and our people don't return. Those moments stay with me.
>
> What strikes me is that many of my colleagues genuinely want to understand our cultural ways, but the systems we work in don't always allow for the relational approaches our communities rely on. These systems were never built with us in mind, and that's where discrimination shows itself, not always through individuals, but through structures that make our people feel unwelcome or unsafe.
>
> That's why I keep coming back to the importance of accomplices. We need non-Aboriginal health professionals who speak up, advocate, and challenge the parts of the system that cause harm. When they put people first and work in ways that centre trust, relationships, and cultural respect, they help create health services where our people can truly belong. (Darlene)

We argue that these accompliceship principles should be part of professional training, informing cultural safety education and used to inform policies and practice.

Accompliceship is an ongoing enacted process (Smith et al., 2016). Health and fitness provide a useful metaphor. We cannot get fitter and healthier by simply *talking* about exercise. Five minutes exercise once a month will have minimal impact. Health and fitness require regular engagement, a resolve to being in relationship with exercise as part of our daily lives. It means persisting even when it's hard or uncomfortable. Health and fitness are affected by everything that has happened in our lives, in our parents', grandparents' and great grandparents' lives, through generationally inherited epigenetics (Grazioli et al., 2017). Health is also impacted by the environment and circumstances we are in right now. We can use this metaphor to think of being an accomplice as equity health and fitness.

All that we do in healthcare is entangled with our attitudes, beliefs, and actions (Gatwiri et al., 2021). We are building professional culture daily, with our actions, communications and relationships. What kind of culture are we generating? We can never underestimate the impact any of us have. An attitude of resolve and determination is needed. Humility in understanding and using our privilege as educated professionals for action will assist us. It can be challenging. We all make mistakes, as we learn and grow new knowledge. Only First Nations people can know if we are culturally safe accomplices (Cargo et al., 2019).

Principle 1: Reality is complex

Professional practice is shaped and influenced by many complex factors (Brown, 2021; 2023). Acknowledging and considering this complexity supports us to refrain from projecting narrow assumptions. We must be open to exploring and learning, treating each person and situation as unique. Thus, within this principle, we explore multiple contexts.

One of the key attributes of life's complexity is diversity. All healthy ecosystems, communities and societies include the contributions of diversity. Working purposefully with cultural diversity can move us beyond reactive unconscious biases and discriminatory practice (Mohamed et al., 2024). Contextual sensitivity can assist us to soften the demands of conformity, opening our curiosity into authentic listening and learning.

Our own context

Our own cultural context and educational experience can create a limited worldview. We are all different, with unique lived experience. Australian education has presented a narrow knowledge focus, obscuring histories and perspectives of non-dominant cultures and peoples (Shay & Wickes, 2017). Acknowledging and exploring the many factors that shape and influence us is a first step to authentic accompliceship.

What has gone before is always impacting the present. We cannot ignore nor deny the personal or collective legacy/s we live with, nor our unique lens on reality, but we can shift the way the past influences the now and our worldview.

Knowing even a little of our own cultural heritage, beyond the 236+ years of non-Indigenous presence in Australia, is an important first step. We are not suggesting complex genealogical research, but having some idea of the story of our family gives a more complete view of generational connection to Australia and Country. This step can begin our own truth telling journey. If our ancestors were involved in actions directly damaging to First Nations peoples, we may need to develop the emotional strength to acknowledge this. Such information can fuel our resolve towards the reciprocity of becoming an accomplice, not out of a sense of being expected to make 'reparations' but out of resolve for a more equitable life for all in the 21st century. With regard to any form of generational violence and trauma, truth telling and peace making are served by affirmative action (Southam, 2021).

This step along the path can provide us with a way to introduce ourselves to First Nations people. Our job title, and or other status information are of little importance in building relationship. The following anecdotes are our own experiences.

> I use self-disclosure and storytelling when building rapport with a new patient/person. Who my family are, where my Country is, exchanging humorous anecdotes. I try to convey with actions and words my accomplice-ship in the struggle for culturally respectful and safe treatment and care. I ask people if they are on their Country, where their mob are from. This exchange of more than clinical information builds relationship and trust. (Elizabeth)

> My introduction to First Nations people often begins with the story of how I inherited my respect for diverse peoples from my mother, and my grandmother. I explain that I am a mother of two daughters and a grandmother to two boys. How long I have lived on Bundjalung Country, and a favourite place that sustains me is something I might share. I always add that I am open to learning. (Shae)

Context of peoples and place

Learning about the history of Peoples/Nations on the Country where we are working and/or living can create respectful engagement with First Nations people. This may include learning the names of different language groups. This information is freely available through the internet and libraries. It is our own responsibility to become informed, rather than the responsibility of First Nations people to bear this cultural load for our benefit (Bruno, 2023). Local First Nations organisations may be readily equipped to provide information, especially once they know that we are sincere in our resolve of accompliceship. These organisations can be useful to connect with along the way.

Wider context

We can also become informed about the current state of affairs for Australian First Nations peoples generally (for example, Chamberlain et al., 2024). First Nations websites provide valuable information. This research can educate about specific topics/issues within your area of practice. Be aware that non-Indigenous sites generally provide information that expresses mainstream/western cultural and political perspectives, omitting truths about the ongoing violence of colonisation and flawed government policies such as the Stolen Generations (Aboriginal & Torres Strait Islander Healing Foundation, 2014; McCallum et al., 2012).

Exploring policies and practices for working with First Nations peoples is important if we are to be informed. All of us are required to operate within policy frameworks; accessing these can assist us to contribute to updating policy and best practice approaches. Such knowledge becomes vital if we experience pushback from colleagues or management. As we advocate for First Nations patients, we may also need to advocate for ourselves, and being well informed will support us.

Acknowledging complex reality relates to our own commitment to make a difference (Brown, 2021). Being clear when articulating the rationale for our journey to becoming accomplices can assist us. We are not suggesting long-prepared speeches/lectures, however a well-honed personal/professional

'position' statement provides a clear, succinct response to questions and challenges. It is also important to regularly recall what motivated us to become accomplices. This is the *why* of the accompliceship journey.

> During my six years of doctoral research, I had my position statement up where I could always see it. It included the words 'I am using my non-Indigenous privilege to contribute to the visibility of Indigenous knowledge in the academy.' This position statement was enacted through always seeking to cite Indigenous scholars at every opportunity (Southam, 2021). I am only one person, but every contribution is valuable. (Shae).

Accompliceship requires retaining the 'big picture' of culturally safe practice, staying informed of policy developments within our disciplines, while remaining responsive to the needs and preferences of First Nations peoples.

Principle 2: Ready to listen and learn

This principle is founded on deep listening and being open to learning (Bruno, 2023). Unbalanced power relationships usually mean the flow of information and dialogue is one way: from practitioner to patient. Listening becomes authentic when we seek to understand the speaker, rather than merely formulating our response/counter point/s. In western culture we are accustomed to listening to respond/debate rather than to deeply understand. Engaging in deep listening and waiting, rather than jumping in as soon as we can, supports relationship. Gaining understanding from First Nations peoples must always be juxtaposed with institutional/policy requirements of our professional role. Integrating First Nations voices within policy and practice requirements is central to being an accomplice (Kirkham, et al., 2019). Deep listening is the basis of authentic, meaningful engagement and relationality with First Nations peoples (Bruno, 2023).

> Talking with an Elder today, I was struck by his ability to truly listen to my words. Then I noticed how he did not speak until he had thought about what I had said, giving me a thoughtful response. (Elizabeth) (Rix et al., 2014)

Without listening, how can we know what is required of us in any role. Resolving to listen and learn is a first step. However, this humble position is not always easy. Being open-minded and allowing our humanity to show develops our knowledge of discrimination within healthcare policy and practice. Accompliceship is not knowledge of fixed outcomes but is non-linear and ongoing. There is ambiguity, no absolute answers and contradiction, because these are qualities of the world we live in. Western knowledge may encourage us to think we can accumulate fixed and absolute knowledge and that as individuals we can own that knowledge. Reality however is more complex. Human knowledge is always partial, always shifting and emerging, with new learning and understanding (Brown, 2021). This is true within medical research, theory and practice.

Principle 3: Taking action

Accomplices may be cautious, fearing getting it wrong or making mistakes. We recommend being brave enough to get it wrong; an inadvertent mistake can lead to learning and relationship. However, taking no action contributes to the complicity of silence (Fredericks et al., 2020). A clear and concise action plan is useful. Aims, goals, actions, and communications relating to our professional role and scope of practice can form a strong professional foundation, providing confidence to act (Bruno, 2023).

Accompliceship actions include standing up against silencing and bullying towards anyone who exposes racism. Such pushback occurs when we expose ourselves within health institutions as someone who will not be complicit or 'white-silenced' about racism and discrimination. White silencing occurs frequently within Australian institutions. It materialises as subtle or blatant workplace lateral violence: i.e. dismissing/denying concerns about racism, scapegoating, gaslighting, bullying, being overlooked for promotion or even the threat of or actual dismissal (Gatwiri et al., 2021)

> I was shocked and appalled at the shift in how staff treated me within a hospital when I outed myself as an active accomplice, I myself became the target of discrimination, a huge learning curve in my journey from ally to accomplice. (Elizabeth)

Rigid power hierarchies are daunting to discover, and workplace bullying is real. Documenting events and experiences is crucial. Our actions are a role model for others. Younger practitioners, peers, and potential accomplices can be positively influenced by our knowledge sharing.

Principle 4: Reflection

Critical reflection is central to being a culturally safe practitioner. Without critically reflecting on our culture and biases, there can be minimal culturally safe practice (Mills et al., 2022). Reflecting on personal/professional experience and learning across our accompliceship journey is crucial (Rix et al., 2024). Being aware of unconscious biases and how these impact our professional practice is key to moving beyond performative allyship (Fredericks et al., 2020; Gatwiri et al., 2021). There may be times of movement and growth, of frustration, perceptions of roadblocks and when things appear to regress. At these times our ongoing critical reflection is of greatest assistance.

A deeper level of learning and understanding results from reflection, enriching our growing knowledge. Keeping a reflective journal supports your accompliceship journey. A newsletter contribution, a presentation, or an article are opportunities to encourage others. Sometimes sharing the accompliceship journey triggers deeper reflection for us and can begin a conscious journey for others. Keeping a reflective journal, also provides supporting evidence in the advent of pushback against our actions as accomplices.

Conclusion

We end here with an invitation to join us on this lifelong journey of accompliceship. The ongoing struggle for practical equity and social justice requires committed practicing accomplices. Our four principles for accompliceship aim to invite and encourage. Knowing that 'Reality is Complex,' being 'Ready to Listen and Learn,' 'Taking Action,' and critical 'Reflection,' can provide a foundational pathway. Practical equity means moving beyond superficial inclusion and box ticking. It is based on authentic relationality with First Nations peoples. Humanity is at a crucial time in history. Social justice and equity are fundamental to a positive future for all. It is time to move beyond being threatened by people who are different from us. Respect for each other and learning to live with cultural diversity are key to a stable and healthy future for all.

Author contributions

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